SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nivolumab, ipilimumab, paclitaxel and carboplatin

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

First-line metastatic squamous NSCLC in adults without sensitising EGFR mutation or ALK translocation. · NCCP 00712 · version 3a · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control metastatic squamous non-small-cell lung cancer, help symptoms and extend life. It is usually given without an expectation of cure.

Schedule
Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation in multiple organs
- ☐ Infusion reactions
- ☐ Delayed effects after treatment stops

Ipilimumab
IMMUNOTHERAPY

- ☐ Immune bowel, liver, skin and hormone inflammation
- ☐ Other serious organ inflammation
- ☐ Infusion reactions

Paclitaxel
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Peripheral neuropathy and hair loss
- ☐ Allergic reactions
- ☐ Muscle and joint pain

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Nausea and vomiting
- ☐ Kidney, hearing and nerve effects
- ☐ Allergic reactions

Discuss treatment aims, alternatives, expected benefit and uncertainty, short-term and lasting harms, and effects on fertility, contraception, pregnancy, work and daily life. Your priorities and questions belong in the decision.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00712 v3a.

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